

Child and adolescent disorders

Neurodevelopmental Disorders



Child and adolescent disorders



Mental retardation or Intellectual Disability



Intellectual Developmental Disorder

- **Intellectual Developmental Disorder** involves below-average intellectual functioning (IQ below 70).



Areas of significant limitations

- communication skills,
- self-care, home living, social or interpersonal skills,
- use of community resources,
- self-direction,
- academic skills,
- work, leisure, health, and safety.



- Normal IQ – 90-110
- Borderline – 71-89



Degrees of retardation

- Mild (IQ 50 to ---70)
- Moderate (IQ 35 to ---50)
- Severe (IQ 20 to ---35)
- Profound (IQ below 20)



Mild Intellectual Developmental Disorder

- **Educable:** can reach up to grade 6
- Learn how to read, write, do arithmetic
- Can be trained for vocational skills
- Can live alone but needs assistance during social and economic stress.



Moderate Intellectual Developmental Disorder

- **Trainable:** can reach up to 2nd grade
- Can learn ADL
- Can be trained in a semi skilled/ unskilled vocational jobs
- Must live in a sheltered environment



Severe Intellectual Developmental Disorder

- Custodial Mental age: 0-2 yrs old
- Barely trainable
- Totally dependent on others
- May not learn to talk & self care



Profound Intellectual Developmental Disorder

- Mental age of young infants
- Require total care through out life



Causes:

- Heredity - such as Tay-Sachs disease or fragile X chromosome syndrome;
- Early alterations in embryonic development such as trisomy 21
- Maternal alcohol intake that causes fetal alcohol syndrome
- Pregnancy or perinatal problems such as fetal malnutrition



Causes

- Hypoxia, infections, and trauma; medical conditions of infancy such as infection or lead poisoning; and environmental influences such as deprivation of nurturing or stimulation.



Signs & Symptoms

- D – decreased Cognitive & intellectual functioning
- D – deficits in psychomotor skills
- D – difficulties in performing self-care activities
- D – degrees of neurologic impairment
- D – depressed or labile mood



Treatment

- 1. BEHAVIOR MANGEMENT
- 2. ENVIRONMENTAL SUPERVISION
- 3. MONITORING OF CHILD'S DEVELOPMENTAL NEEDS & PROBLEMS
- 4. PROGRAMS THAT MAXIMIZE SKILLS
- 5. FAMILY THERAPY



Nursing Intervention

- 1. Determine a plan of care
- 2. Monitor child's developmental levels
- 3. Teach about natural & normal feelings of emotions
- 4. Safety needs
- 5. Prevent self-injury
- 6. Behavior modification
- 7. KEY COMMUNICATION : brief, simple & consistent



Learning Disorder

- Is diagnosed when a child's
- achievement in **reading, mathematics, or written expression** is below that expected for age.
- Reading and written expression disorders usually are identified in the first grade;



Learning Disorder

- **Math** disorder may go undetected until the child reaches fifth grade.
- Low self-esteem and poor social skills are common in children with L.D.
- As adults, some have problems with employment or social adjustment; others have minimal difficulties.



MOTOR SKILLS DISORDER

- *Developmental coordination disorder*
- Is impaired coordination severe enough to interfere with academic achievement or activities of daily living.
- Sensory integration programs are specific physical therapies prescribed to target improvement in areas where the child has difficulties.



- **Stereotypic movement disorder** is characterized by rhythmic, repetitive behaviors, such as hand waving, rocking, head banging, and biting, that
- appears to have no purpose



COMMUNICATION DISORDERS

- is diagnosed when a communication deficit is severe enough to hinder development , academic achievement, or ADLs including socialization.



COMMUNICATION DISORDERS

- Includes:
- *Expressive language disorder*
- *Mixed receptive expressive language disorder.*
- *Phonologic disorder*
- *Stuttering*



COMMUNICATION DISORDERS

Includes:

- Speech sound disorder
- Social communication disorder



Nursing Intervention

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5. Prevent self injury
6. Behavior modification
7. KEY COMMUNICATION – brief, simple and consistent



Elimination Disorders

- **Encopresis**- the repeated passage of feces into inappropriate places.
- **Intentional encopresis** is often associated with oppositional defiant disorder (ODD) or conduct disorder.
- **Enuresis**- the repeated voiding of urine during the day or at night into clothing or bed by a child at least 5 years of age.



Elimination Disorders

- **Sluggish cognitive tempo (SCT)** - syndrome that is not a DSM-5-TR diagnosis



PERVASIVE DEVELOPMENTAL DISORDERS

Includes:

- Autistic disorder (classic autism) or Autism spectrum disorder (ASD) Rett's disorder,
- Childhood disintegrative disorder, and
- Asperger's disorder.



Autistic Disorder/Autism spectrum disorder (ASD)



Autistic Disorder

- More prevalent in boys
- Present by age 3, dx.
- Dx. Is made when child is 18-3yrs.



Autistic Disorder

- Child has little eye contact,
- few facial expressions,
- does not communicate verbally or with gestures,
- doesn't relate to peers or parents,
- lacks spontaneous enjoyment;
- apparent absence of mood and affect;
- cannot engage in play or make-believe with toys



BOX 22.1

Behaviors Common With ASD

- Not responding to own name by 1 year (e.g., appears not to hear)
- Doesn't show interest by pointing to objects or people by 14 months of age
- Doesn't play pretend games by 18 months of age
- Avoids eye contact
- Prefers to be alone
- Delayed speech and language skills
- Obsessive interests (e.g., gets stuck on an idea)
- Upset by minor changes in routine
- Repeats words or phrases over and over
- Flaps hands or rocks or spins in a circle; answers are unrelated to questions
- Unusual reactions to sounds, smells, or other sensory experiences



Autistic Disorder

Stereotyped body movement:

- Hand-flapping, body-twisting, head-banging
- Autism may improve, sometimes substantially, as language and communication skills are learned.
- Traits persist into adulthood. Few attain complete independence, marry, or have children.



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DSM-5 CRITERIA FOR AUTISM SPECTRUM DISORDER

- A. Social Communication and Interaction (Must have 3 of 3)
 - 1. Deficits in social-emotional reciprocity
 - 2. Deficits in nonverbal communicative behaviors used for social interaction
 - 3. Deficits in developing, maintaining, and understanding social relationships



DSM-5 CRITERIA FOR AUTISM SPECTRUM DISORDER

- B. Restricted, repetitive patterns of behavior, interests, and activities
- (Must have 2 of 4)
- 1. Stereotyped or repetitive motor movements, use of objects, or speech
- 2. Insistence on sameness, inflexible adherence to routines, or ritualized patterns of verbal or
- nonverbal behavior
- 3. Highly restricted, fixated interests that are abnormal in intensity or focus
- 4. Hyper- or hyporeactivity to sensory input or unusual interest in sensory aspects of the
- environment



Autistic Disorder

- Most autistic children are mainstreamed in school.
- Medications may be used to target specific behaviors but do not treat the autism.
- Goals are to reduce behavioral symptoms and promote learning, development, and language skills.



- PECULARITIES :
- Gaze
- Hearing
- Play



Diagnostic Test

1. Checklist for Autistic Children
2. Autism Screening Questionnaire
3. Autism behavior checklist
4. Childhood Autism Rating Scale



Treatment

- 1. Early intervention, special education (language therapy) & family support.
- 2. Drugs, Methylphenidate – to reduce inattentiveness, impulsivity & overactivity.



Treatment

3. Haloperidol & Risperdal – to treat aggressive behavior, self injury

- 4. Family counselling



Nursing Intervention

1. Promote Safety
2. Communication
3. Help Child establish- self boundaries
4. Maintain consistency
5. Adopt Techniques



Nursing Intervention

- 6. Choose words carefully when speaking.
- 7. Provide emotional support to parents
- 8. Promote communication : close, face-to-face contact with the child



Rett's Disorder



Rett's Disorder

- Similar to autism but occurs only in girls; symptoms worsen as the child grows older.
- Characterized by the development of multiple deficits after a period of normal functioning.



Asperger's Disorder



Asperger's Disorder

- Same impaired social interaction and restricted stereotyped movements as in autism, but no language or cognitive delays; rare

Cause: Due to abnormalities in brain functions and structure

- Hans Asperger (1944) Viennese pediatrician



CHILDHOOD DISINTEGRATIVE DISORDER

- marked regression in multiple areas of functioning after at least 2 years of apparently normal growth and development.
- Regression in language and social skills, bowel and bladder control



Attention Deficit Hyperactivity Disorder (ADHD)



Attention Deficit Hyperactivity Disorder (ADHD)

- Marked by developmentally inappropriate inattention, impulsiveness, and in some cases, HYPERACTIVITY (Overactivity)



Risk factors for ADHD

- family history of ADHD; male relatives with antisocial personality disorder or alcoholism; female relatives with
- somatization disorder; lower socioeconomic status; male gender; marital or
- family discord, including divorce, neglect, abuse, or parental deprivation;
- low birth weight; and various kinds of brain insult (Black & Andreasen, 2021).



- The Child Behavior Checklist, Teacher Report Form, and Youth Self Report (for ages 11–18 years) are rating scales frequently used to determine problem areas and competencies.
- The ADHD-FX has also proved to be a valid, reliable, and culturally appropriate measure of functional impairment of at-risk students



Signs & Symptoms

- INATTENTION
 - *Fails to close attention details
 - - Has trouble sustaining attention in tasks
 - Not to listen when spoken directly
 - Fails to follow instructions
 - Trouble in organizing tasks
 - Reluctant to engage in tasks
- HYPERACTIVITY-IMPULSIVITY
 - Fidgets with his hands or feet or squirms in his seat
 - Run abouts
 - Trouble in playing & engaging in leisure activities
 - “On the go”, “into everything”



ONSET

- Diagnosed when the child began pre school or school age



Treatment

- Goals of treatment involve managing symptoms,
- Reducing hyperactivity, impulsivity, and increasing the child's attention



Treatment

- Combination of pharmacotherapy with behavioral, psychosocial, and educational interventions.
- sugar-controlled diets and megavitamin therapy.



PSYCHOPHARMACOLOGY

- 1. (Ritalin) and an amphetamine compound(Adderall)
- 2. Methylphenidate
- 3. Dextroamphetamine(Dexedrine) and pemoline (Cylert)
- 4. Antidepressants



Nursing Interventions

1. Ensuring the child's safety and that of others
2. Improved role performance
 - Give positive feedback
3. Simplifying instructions/directions
4. Structured daily routine
 - Establish a daily schedule.
- 5. Client/family education and support



Conduct Disorder



Conduct Disorder

- Persistent antisocial behavior of children and adolescents that significantly impairs ability to function in social, academic, or occupational areas



- Symptoms are clustered in four areas:
- Aggression to people and Animals,
- Destruction of property, deceitfulness
- Theft, and serious violation of rules



SYMPTOMS OF CONDUCT DISORDER

- Aggression to people and animals
- Bullies, threatens, or intimidates others
- Physical fights
- Use of weapons
- Forced sexual activity
- Cruelty to people or animals
- Destruction of property
- Fire setting



SYMPTOMS OF CONDUCT DISORDER

- Vandalism
- Deliberate property destruction
- Deceitfulness and theft
- Lying
- Shoplifting
- Breaking into house, building, or car
- Cons others to avoid responsibility
- Serious violation of rules



SYMPTOMS OF CONDUCT DISORDER

- Stays out overnight without parental consent
- Runs away from home overnight
- Truancy from school



Signs & Symptoms

- C – Cheats
- O – Obnoxious
- N – Nasty in speech & behavior
- D – Drug & alcohol use
- U – unpredictable behavior
- C – cruel to people or animals
- T – truant
- D – destroys property
- D – destroys property
- I - intimidates
- S – Steals
- O – onset of antisocial PD
- R - rages
- D – disrespectful
- E – esteem low
- R – reckless or risky



ONSET

- **Childhood-onset type** - involves symptoms before 10 years of age
- **Adolescent-onset type** - is defined by no behaviors of conduct disorder until after 10 years of age.



Classification

Conduct disorders can be classified as:

- Mild,
- Moderate,
- Severe



Etiology

- Genetic vulnerability,
- Environmental adversity,
- Poor coping



Risk Factors

- Poor parenting,
- low academic achievement,
- Poor peer relationships,
- Low self-esteem



Treatment

- 1. Psychotherapy
- 2. Drugs that can treat neurologic difficulties e.g. **Antipsychotic** if child presents a clear danger to others
- lithium ,carbamazepine (Tegretol) or valproic acid (Depakote) –labile mood
- 3. Educational Strategies



Nursing Intervention

1. Decreasing violence and increasing compliance with treatment
 - Protect others from client's aggression and manipulation.
 - Set limits for unacceptable behavior.
 - Provide consistency with client's treatment plan.
 - Use behavioral contracts.



Nursing Intervention

2. Improving coping skills and self-esteem

- Show acceptance of the person, not necessarily the behavior.
- Encourage the client to keep a diary.
- Teach and practice problem-solving skills.



Nursing Intervention

3. Promoting social interaction

- Teach age-appropriate social skills.
- Role-model and practice social skills.
- Provide positive feedback for acceptable behavior.

4. Providing client and family education



Oppositional Defiant Disorder



Oppositional Defiant Disorder

- Enduring pattern of uncooperative, defiant, and hostile behavior toward authority figures that does not involve major antisocial violations



Signs & Symptoms

- 1. Loses TEMPER
- 2. often argues with adults
- 3. Often defies or refuses to comply with adult's requests
- 4. Annoys people deliberately
- 5. blames others for mistakes
- 6. Touchy or easily annoyed by others
- 7. Angry & resentful
- 8. Spiteful & vindictive



Treatment & Nursing Intervention

- FAMILY THERAPY
- Convey acceptance
- Discuss child's limits & consequences
- Help address negative feelings
- Stop plotting – accept responsibility of own behavior
- Role-playing – problem solving skills



FEEDING AND EATING DISORDERS OF INFANCY AND EARLY CHILDHOOD

- **PICA** - persistent ingestion of nonnutritive substances
- **RUMINATION DISORDER** - repeated regurgitation and rechewing of food.
- **FEEDING DISORDER** - is
- characterized by persistent failure to eat adequately, which results in significant weight loss or failure to gain weight.



Tic Documentary



TIC DISORDERS

- A tic is a sudden, rapid, recurrent, non-rhythmic, stereotyped motor movement or vocalization.



Tourette's Disorder



Tourette's Disorder

- Multiple motor tics and one or more vocal tics; vocal tics can be name-calling or profanity; can persist into adulthood
- Person is embarrassed and self-conscious, and has significant impairment in academic, social, occupational areas.



Signs & Symptoms

- Tics
 - eye blinking
 - Facial grimacing
 - Neck, head jerking
 - Neck stretching
 - Foot stamping
 - Body twisting & bending



Treatment

- 1. Psychotherapy
- 2. Pharmacotherapy
 - Low dose anti-psychotics, PIMOZIDE, RESPERIDONE, HALOPERIDOL
 - SSRIs, fluoxetine
 - Anti-HPN, clonidine
- NURSING INTERVENTION: Develop child's TRUST; provide quiet activities; Stay positive about himself



Chronic Motor or Tic Disorder



Chronic Motor or Tic Disorder

- Has only vocal tic or only motor tics, not both, as with Tourette's.



ELIMINATION DISORDERS

- **Encopresis** - is the repeated passage of feces into inappropriate places such as clothing or the floor, by a child who is at least 4 years of age.



ELIMINATION DISORDERS

- **Enuresis** - is the repeated voiding of urine during the day or at night into clothing or bed by a child at least 5 years of age.



Self-Awareness Issues

- Recognize own beliefs about parenting and how they differ from others.
- Focus on patient's strengths, not just problems.
- Try to have positive impact on child, even when disability is severe.
- Support parents



Thank you!

